

NORCUTIN TABLET 5MG

DESCRIPTION: A 7 mm round, scored tablet, green in colour with markings 'DUO 861' on one side and a 'star' on the reverse side.

COMPOSITION: NORCUTIN TABLET 5MG: Each tablet contains Norethisterone 5 mg.

PHARMACODYNAMICS: Norethisterone is progestogens derived from nortestosterone that have weak oestrogenic and androgenic properties.

PHARMACOKINETICS: Norethisterone is absorbed from the gastrointestinal tract, undergoing first-pass hepatic metabolism, with peak plasma concentrations occurring 1 to 2 hours after a dose by mouth. It exhibits biphasic pharmacokinetics; an initial distribution phase is followed by a prolonged elimination phase with a half-life of about 8 hours or more. Norethisterone is highly protein bound; about 60% to albumin and 35% to sex hormone binding globulin. Administration with an oestrogen increases the proportion bound to sex hormone binding globulin. It is metabolized in the liver with 50% to 80% of a dose being excreted in the urine and up to 40% in the faeces.

INDICATION: Dysfunctional bleeding, primary and secondary amenorrhoea, premenstrual syndrome, mastopathy, timing of menstruation, endometriosis, adjunct to oestrogen hormone replacement therapy.

RECOMMENDED DOSAGE:

Before starting Norethisterone a thorough general medical and gynaecological examination (including the breasts) should be carried out and pregnancy must be excluded. As a precaution, control examinations should be conducted at intervals of about 6 months during long-term treatment with Norethisterone. The tablets are to be swallowed whole with some liquid. Unless otherwise prescribed by the doctor, the following dosages are recommended:

Dysfunctional bleeding: The administration of 1 tablet Norethisterone 3 times daily over 10 days leads to the arrest of uterine bleeding not associated with organic lesions within 1-3 days. In individual cases, bleeding diminishes during the first few days after the commencement of tablet-taking and does not stop until about 5 days later. For the treatment to be successful, Norethisterone administration should be continued regularly even after arrest of bleeding (up to a total of 30 tablets). About 2 - 4 days after discontinuation of treatment a withdrawal bleeding will occur resembling a normal menstruation in intensity and duration.

Slight bleeding during tablet-taking: Occasionally, slight bleeding may occur after initial arrest of bleeding. In these cases tablet-taking must not be interrupted.

Missing arrest of haemorrhage, heavy breakthrough bleeding: Norethisterone can give valuable hints concerning differential diagnosis of uterine bleeding. If the bleeding does not stop in spite of regular tablet-taking, an organic cause must be considered. The attending physician must be informed immediately, because further measures are then mostly required. This applies also in cases where after initial arrest of haemorrhage, heavier bleedings still occur during tablet-taking.

Prevention of recurrence: To prevent recurrence of dysfunctional bleeding, it is recommended to administer Norethisterone prophylactically during the next three cycles, i.e. 1 tablet Norethisterone 2-3 times daily from the 19th to the 26th day of the cycle (1st day of the cycle - 1st day of the last bleeding). The withdrawal bleeding occurs some days after administration of the last tablet. Only the physician can decide whether this measure is necessary. His decision is then based on the course of the basal body temperature, which must be measured daily.

Primary and secondary amenorrhoea: In the case of secondary amenorrhoea hormone treatment is to be given at the earliest 8 weeks after the last menstrual period.

Premenstrual syndrome, mastopathy: Premenstrual symptoms such as headaches, depressive moods, water retention, a feeling of tension in the breasts may be relieved or palliated by 1 tablet Norethisterone 2-3 times daily from the 19th to the 26th day of the cycle.

Timing of menstruation: The monthly bleeding can be advanced or postponed if particular circumstances require this. However, advancement with progestogen-estrogen combinations is definitely to be preferred, because the occurrence of a pregnancy is virtually ruled out by the inhibition of ovulation. As opposed to this, the postponement of menstruation calls for the use of Norethisterone at a time when the necessary exclusion of pregnancy can be problematical since Norethisterone must be given at a time when pregnancy cannot be excluded using the currently available examination methods. Therefore, this method remains restricted to those cases in which there is no possibility of early pregnancy in the cycle concerned.

Dosage: 1 tablet 3 times daily for not longer than 10-14 days, beginning about 3 days before the expected menstruation. Bleeding will occur 2-3 days after having stopped medication. If it does not, the doctor must be consulted.

Endometriosis: Treatment is commenced on the 5th day of the cycle with 1 tablet twice daily, increasing to 2 tablets twice daily in the event of spotting. When the bleeding ceases, the initial dose can be resumed. Duration of treatment at least 4-6 months. During treatment, ovulation and menstruation do not occur. After discontinuation of hormone treatment a withdrawal bleeding will occur.

ROUTE OF ADMINISTRATION: Oral

CONTRACINDICATIONS: Severe disturbances of liver function, Dubin-Johnson syndrome, Rotor syndrome, previous or existing liver tumours, a history of jaundice or severe pruritus during pregnancy, a history of herpes of pregnancy, thromboembolic processes.

WARNINGS & PRECAUTIONS: Prior to initiation of therapy in women, a physical examination should be performed, including special attention to the breasts and pelvic organs and a Papanicolaou test (Pap smear). Should be used with caution in patients with a history of mental depression, and during pregnancy. Contraindicated in patients with thrombophlebitis, thromboembolic disorders, cerebral apoplexy or a history of these conditions, asthma or epilepsy or other conditions affected by fluid retention. The drug is also contraindicated in patients with undiagnosed vaginal bleeding, missed abortion, markedly impaired liver function or liver diseases, or carcinoma of the breast or for use as a pregnancy test.

INTERACTION WITH OTHER MEDICAMENTS:

Norethisterone with cyclosporin, Danazol – Blood level of cyclosporin increase when either danazol or norethisterone is taken.

Norethisterone with Rifampicin – Plasma half-life and levels of norethisterone reduced significantly by rifampicin.

USE IN PREGNANCY & LACTATION:

Use in Pregnancy: Hypospadias in males and virilisation of females has been reported in the off spring of mothers who have taken norethisterone during pregnancy.

Use in Lactation: Norethisterone is contraindicated in pregnancy, and should be avoided during lactation, as it is present in breast milk

SIDE EFFECTS:

Spotting, amenorrhoea, changes in cervical erosion and secretions, edema, weight gain or loss, mental depression, thrombophlebitis, pulmonary embolism, cerebral thrombosis, embolism

SYMPTOMS AND TREATMENT OF OVERDOSE:

Overdosage may be manifested by nausea, vomiting, breast enlargement and later vaginal bleeding. There is no specific antidote and treatment should be symptomatic.

Gastric lavage may be employed if the overdosage is large and the patient is seen sufficiently early (within four hours)

STORAGE CONDITIONS: Store below 30°C. Protect from light. Keep out of reach of children. Jauhkan daripada kanak-kanak.

SHELF LIFE: Please refer to outer package.

PACK SIZE: Pack in blister pack of 3 x 10's & 50 x 10's tablets in a box

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

Lot 2599, Jalan Seruling 59 Kawasan 3,

Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA.